

Improvement Portfolio Review — Q1 FY2027

Quarterly executive portfolio review, September 22, 2026. Prepared by Hannah Mercer, Director, Performance Improvement. Status as of the September 18 milestone tracker.

Portfolio at a glance

Four active projects; the imaging scheduling consolidation closed in April 2025. Of 34 tracked milestones, 21 are complete, 6 on track, 3 not started, 2 late, 1 at risk and 1 pending a decision.

West patient flow pilot: in evaluation

Central Staffing Phase 2: behind on self-scheduling

CMMS consolidation: on track

Fort Bend fourth floor: on track for the February board

West patient flow pilot

Discharges before 11 a.m. on the three pilot units rose from 14.3% in February to 27.1% from March through August. Monthly results ranged from 23.4% to 29.7%; the 30% target was not met in any of the six months. Boarding fell, but other West units fell by a similar share over the same season.

Pilot is in evaluation; spread decision at the October operating committee

Flow checkpoint recommendation

Continue the pilot unchanged through December 2026

Spread to Progressive Care and 6 West in January 2027 if approved

Keep the 30% target

Add a 10 a.m. discharge-readiness call between case management and hospitalists

Central Staffing Phase 2

Runs January 2026 to March 2027. Hospital Fort Bend joined in February. The float pool has grown from 38 RN FTEs at Phase 1 go-live to 47 filled against a target of 52.

Self-scheduling configuration about six weeks behind plan; forecast September 25

Outpatient onboarding moved to October 5

Two-unit self-scheduling pilot at risk: forecast November 16 against October 1

CMMS consolidation

Approved in June. Data cleanup runs July to October; legacy migration January to April 2027. The September 16 requirements review decided on one enterprise CMMS instance with separate clinical engineering work order types, and the enterprise asset tag format with legacy tags kept as an alternate identifier.

Open: replace AEM category defaults with device-level risk scores?

Open: how surgery center devices are handled under the services agreement

Fort Bend fourth floor

The options memo asks for \$350,000 for schematic design of both build-out scopes, so estimates can be refined before the board decides in February 2027 as part of the FY2028 capital plan. Executive options review is scheduled for October 6.

Closed: imaging scheduling

Pavilion and Stafford imaging scheduling moved to central scheduling between October 2024 and March 2025, and the project closed in April 2025. Therapy scheduling followed in FY2026 as planned.

Decisions for the executive team

Flow pilot spread: October operating committee

Approve \$350,000 for fourth-floor schematic design

Accept revised self-scheduling dates or add configuration support

Give direction on the two open CMMS questions before configuration starts